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Project #14: Septic Shock Mortality and Sepsis Readmission Reduction

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AIM

The aim of this project is to reduce septic shock mortality rates to 28.55% and sepsis hospital readmissions to 13.75% through calendar year 2023, due to finishing above goal for both of these outcome metrics in calendar year 2022, for our sepsis patient population.

Current State (Plan)

- We ended calendar year 2022 with a higher septic shock mortality rate than our goal of 28.55%
- Current mortality goal set by as both an internal target and a system-wide goal
- We ended calendar year 2022 with a higher sepsis readmission rate than our goal of 13.75%
- Current readmission goal is set as an internal target for our facility, based on .

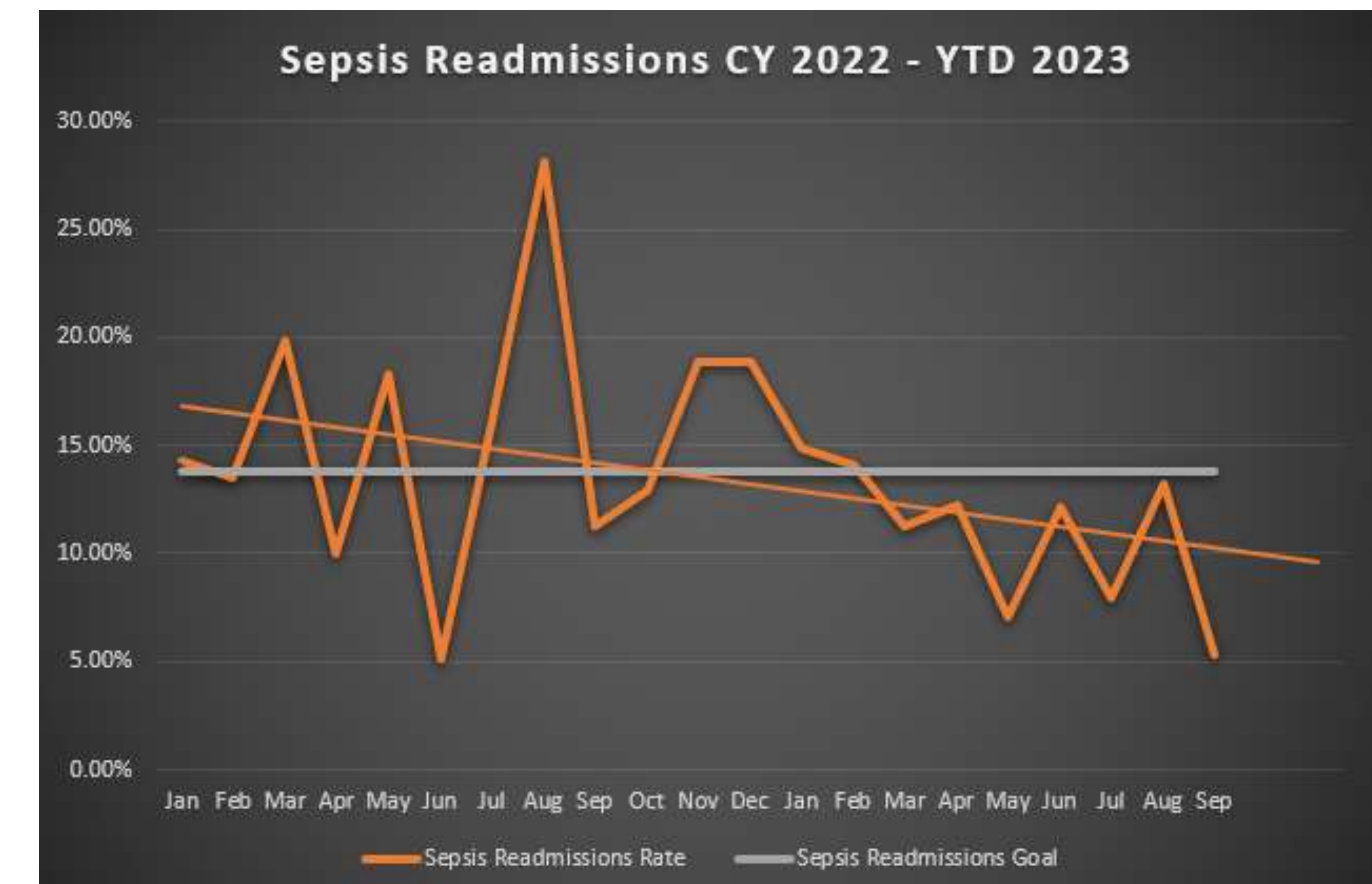
Interventions

- Use mortality data from Premier to determine a realistic goal for improving septic shock related mortality.
- Implementation of an electronic Sepsis Narrator tool for eliminating waste and improving inter-departmental communication.
- Implementation of a robust sepsis response protocol to enhance early identification and treatment.
- The reinstatement of a comprehensive sepsis education packet.
- Targeted case reviews for both septic shock mortalities and sepsis readmissions.
- Tracking and trending of various process metrics that are aimed to improve patients' outcomes. Two of the tracked metrics are:
 - Sepsis readmissions that had a follow-up appointment scheduled at time of index discharge.
 - Appropriate placement of sepsis patients on sepsis unit at time of admission.

Evaluation of Interventions

- Manual review of all sepsis rapid responses, looking at sepsis BPA times, compliance with the SEP-1 bundle and percentage of sepsis rapid responses that ended up with sepsis diagnoses to ensure the process is working effectively.
- Review mortality rates at each Henry Ford Jackson Sepsis Steering Committee meeting.
- Manually review each septic shock mortality cases, looking compliance with SEP-1 and areas for improvement. These cases were thoroughly reviewed and discussed at different sepsis workgroups.
 - POA-Y cases reviewed/discussed at the emergency department sepsis workgroup meeting.
 - POA-N cases reviewed/discussed at the ICU and Med-surg sepsis workgroup meetings.
- Monthly reviewing sepsis related readmissions to see if there were opportunities for improvement. Looking closely at patients that discharge home to see if they had a primary care provider follow-up appointment scheduled within 14 days of discharging from the hospital.
- Keeping track of how many readmissions are being readmitted from subacute rehabs.
- Reviews being done three times per week looking over the patients on the sepsis unit to see if they have an infection/sepsis diagnosis. As well as looking at all the other sepsis patients that were placed somewhere else in the hospital to see if they were a candidate for the sepsis unit. This report is sent to manager/directors of the respective areas to assist with breakdown of potential barriers for continued improvement.

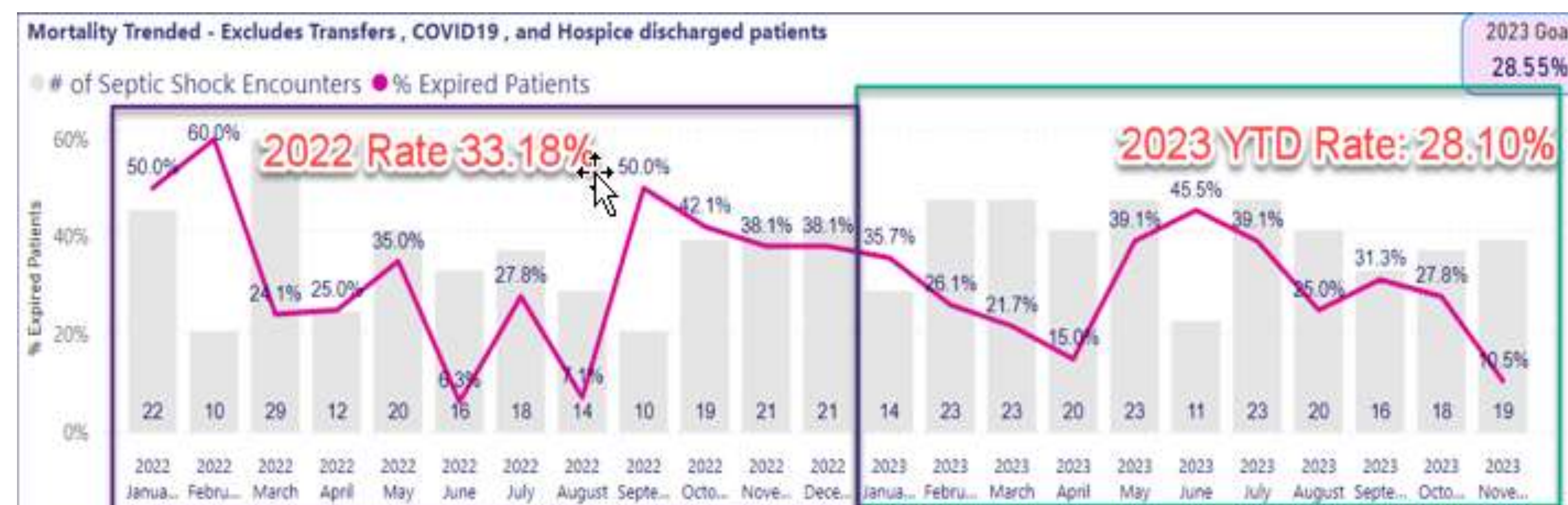
Results cont...



Sustain and Spread

- Conducting annual sepsis education for nurses, physicians and physician residents
- Maintaining continuous surveillance of defined process metrics
- Establishing an open forum for staff feedback on sepsis processes
- Actively involving frontline staff in decisions impacting their workflows

Results



Keys to Success

- Hospital leadership involvement in the program is vital to driving engagement and change
- Keep goals SMART
- Keep the number of goals manageable (suggest 3-5)
- Revise goals when necessary
- Don't work on too many things at one time
- Share data with bedside staff
- Celebrate successes!